A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

16 April 2026

Dr Robin Wong
Turn The Corner

Dear Dr Wong,

Re: Mrs Jean James
DOB: 1945-06-07
Address: 158 Elm Street, NORTHCOTE, VIC, 3070

Summary

- **Unexplained iron deficiency and chronic constipation.**
- **For gastroscopy and colonoscopy under sedation.**
- **No action required by you.**

I had the pleasure of conducting an in-person consultation with Jean James, thank you for referring her.

Symptoms and history of presenting complaint Jean was referred for investigation of a new diagnosis of iron deficiency. She is currently asymptomatic from a gastrointestinal perspective regarding upper tract symptoms, denying any history of dyspepsia, dysphagia, or haematemesis. She noted that her stools have been black recently, though this coincides with the commencement of oral iron supplementation.

Regarding her bowel habit, Jean reports a 12-month history of significant constipation. She often goes up to eight days without a bowel movement and describes passing hard, pellet-like stools. This is associated with significant straining and occasional scant bright red blood on the paper, which is likely secondary to haemorrhoidal irritation or a small anal fissure. She denies any significant weight loss or family history of bowel cancer.

Diet Jean maintains a reasonable intake of red meat, including pork and beef. Her alcohol intake is moderate, involving approximately one bottle of wine three to four times per week.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Past medical history and medications Jean has a complex medical history. She has a background of cirrhosis secondary to Hepatitis C, which was successfully treated with a previous course of interferon and subsequently modern direct-acting antivirals; she remains under annual surveillance with the Alfred Hospital liver clinic. In late 2023, she underwent a wedge resection for a small lung cancer and has a history of mild emphysema. Her cardiovascular history is significant for peripheral vascular disease requiring stents approximately six months ago. Her current medications include aspirin 100mg daily, telmisartan, rosuvastatin, and Coloxyl. Notably, she was prescribed clopidogrel following her vascular procedure but elected to cease this prematurely.

Social history, family history Jean lives alone, as her husband has recently moved into aged care, though she has good support from her daughter. She is a current smoker, smoking 20 cigarettes per day. There is no known family history of colorectal malignancy or inflammatory bowel disease.

Examination On examination, Jean appeared well. Her abdomen was soft and non-tender with no palpable masses or organomegaly. I did not appreciate any stigmata of chronic liver disease on this occasion. Her weight has remained stable.

Impression and plan The etiology of Jean's iron deficiency remains unclear. While it is possible that her recent major thoracic and vascular surgeries, combined with the commencement of aspirin, have contributed to her low iron stores, we must exclude occult gastrointestinal malignancy or malabsorption. Her chronic constipation also warrants further evaluation, particularly given the change in bowel habit over the last year.

I have recommended that she undergo a gastroscopy and colonoscopy. We discussed the procedural risks, specifically the small risk of perforation and post-polypectomy bleeding. Given her recent vascular stenting, the risk of stent thrombosis outweighs the risk of procedural bleeding; therefore, I have instructed her to continue her aspirin throughout the perioperative period. We will arrange these procedures, and I will review her with the histology results in due course.

Thank you again for referring this patient.

Yours sincerely,

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